

Amendment

This Amendment (this “Amendment”) is to the Medical Group Contract, effective as of 02/15/2013 (the “Agreement”), between UnitedHealthcare Insurance Company, UnitedHealthcare of Washington, Inc., UnitedHealthcare of Oregon, Inc. and PacifiCare Health and Life Insurance Company, each contracting individually (each, “United”), and Radiantcare Radiation Oncology (“Medical Group”).

This Amendment is effective on 02/15/2025 (the “Amendment Effective Date”).

The parties agree to modify the Agreement as follows:

1. The capitalized terms used in this Amendment, but not otherwise defined, will have the meanings ascribed to them in the Agreement. If UnitedHealthcare of Oregon, Inc. is not listed as a party to the Agreement, the definition of “United” is revised to include “UnitedHealthcare of Oregon, Inc.” and UnitedHealthcare of Oregon, Inc. is added as a party to the Agreement.
2. The term of the Agreement is amended to have a new initial term beginning as of the Effective Date of the Agreement and ending on 2/14/2028. This agreement will thereafter renew automatically for renewal terms of one year.
3. Each payment appendix listed below is deleted in its entirety: Payment Appendix – All Payer
4. Each payment appendix attached to this Amendment is added to the Agreement: Payment Appendix – All Payer
5. Except as expressly provided in this Amendment, the terms and conditions of the Agreement remain in full force and effect. If the provisions of this Amendment conflicts with the provisions of the Agreement, the terms and conditions of this Amendment control.

UnitedHealthcare Insurance Company, UnitedHealthcare of Washington, Inc., UnitedHealthcare of Oregon, Inc. and PacifiCare Health and Life Insurance Company, each individually		Radiantcare Radiation Oncology, as signed by its authorized representative	
Signature:		Signature:	
Print Name:	Scott Williams	Print Name:	Rebecca Gilbert
Title:	VP, Network Management	Title:	Practice Manager
Date:	1-2-2025	Date:	12-31-2024
		TIN:	91-2018691

Payment Appendix - All Payer

All Payer Fee Information Document: WA 95404/95405

Unless another Payment Appendix to this Agreement applies specifically to a particular Benefit Plan as it covers a particular Customer, the provisions of this Payment Appendix apply to Covered Services rendered by Medical Group to Customers covered by Benefit Plans sponsored, issued or administered by all Payers.

This Appendix applies to the following networks:

- ☒ Charter
- ☒ Choice
- ☒ Core
- ☒ Navigate
- ☒ Options
- ☒ SignatureValue
- ☒ Select

Payment Appendix

All Payer Fee Information Document YEAR 1

Fee Schedule Specifications for OTHER RADIOLOGY: as of 2/1/2025
Report Date: 12/30/2024

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Type Of Service Description	Primary Fee Source	Pricing Level
EVALUATION & MANAGEMENT	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
EVALUATION & MANAGEMENT - NEW PT SF MDM	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
EVALUATION & MANAGEMENT - NEW PT LOW MDM	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
EVALUATION & MANAGEMENT - NEW PT MOD MDM	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
EVALUATION & MANAGEMENT - NEW PT HIGH MDM	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
EVALUATION & MANAGEMENT - EST PT NOT REQ PHYS QHP	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
EVALUATION & MANAGEMENT - EST PT SF MDM	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
EVALUATION & MANAGEMENT - EST PT LOW MDM	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
EVALUATION & MANAGEMENT - EST PT MOD MDM	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
EVALUATION & MANAGEMENT - EST PT HIGH MDM	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
EVALUATION & MANAGEMENT - NEONATAL	2020 CMS RBRVS Carrier Locality (0000000)	149.000%
EVALUATION & MANAGEMENT - PREVENTIVE	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
EVALUATION & MANAGEMENT - NURSING FACILITY SVCS	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
E&M - EMERGENCY MEDICINE DEPARTMENT VISIT	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - INTEGUMENTARY	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - MUSCULOSKELETAL	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - RESPIRATORY	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - CARDIOVASCULAR	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - HEMIC & LYMPHATIC	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - MEDIASTINUM & DIAPHRAGM	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - DIGESTIVE	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - URINARY	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - MALE GENITAL	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - FEMALE GENITAL	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - MATERNITY & DELIVERY	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - ENDOCRINE	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - NERVOUS	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - EYE & OCULAR ADNEXA	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
SURGERY - AUDITORY	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
RADIOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
RADIOLOGY - BONE DENSITY	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
RADIOLOGY - CT	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
RADIOLOGY - MAMMOGRAPHY	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
RADIOLOGY - MRI	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
RADIOLOGY - MRA	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
RADIOLOGY - NUCLEAR MEDICINE	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
RADIOLOGY - PET SCANS	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
RADIOLOGY - RADIATION THERAPY	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
RADIOLOGY - ULTRASOUND	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
LAB - PATHOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	60.000%
OFFICE LAB	2020 CMS Clinical Lab Schedule National Limit	60.000%
CLINICAL LABORATORY	2020 CMS Clinical Lab Schedule National Limit	42.000%
MEDICINE - OPHTHALMOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	125.000%
MEDICINE - CARDIOVASCULAR	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
MEDICINE - ALLERGY & CLINICAL IMMUNOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
MEDICINE - CHIROPRACTIC MANIPULATIVE TREATMENT	2020 CMS RBRVS Carrier Locality (0000000)	90.000%
MEDICINE - PHYSICAL MED AND REHAB - MODALITIES	2020 CMS RBRVS Carrier Locality (0000000)	90.000%
MEDICINE - PHYSICAL MED AND REHAB - THERAPIES&OTHER	2020 CMS RBRVS Carrier Locality (0000000)	90.000%
MEDICINE - ENTERAL FORMULA	2020 UHC PEN NonRural Floor	40.000%
MEDICINE - OTHER	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
MEDICINE - IMMUNIZATION ADMINISTRATION	2020 CMS RBRVS Carrier Locality (0000000)	100.000%
MEDICINE - CHEMO ADMIN	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
OBSTETRICS - GLOBAL	2020 CMS RBRVS Carrier Locality (0000000)	226.991%
IMMUNIZATIONS	UHC Immunization Fee Schedule	113.000%
INJECTABLES/OTHER DRUGS	CMS Drug Pricing	100.000%
INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	CMS Drug Pricing	100.000%
INJECTABLES - IVIG	CMS Drug Pricing	100.000%
INJECTABLES-ORIGIN/BIOSIMILAR NON-ONCOLOGY DRUGS	CMS Drug Pricing	100.000%
INJECTABLES-ORIGIN/BIOSIMILAR ONCOLOGY DRUGS	CMS Drug Pricing	100.000%
INJECTABLES-SALINE & DEXTROSE SOLUTIONS	CMS Drug Pricing	100.000%
DME & SUPPLIES	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - RESPIRATORY	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - ORTHOTICS	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - PROSTHETICS	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - OSTOMY	2020 CMS DME NonRural Floor	40.000%
AMBULANCE	2020 CMS Ambulance Schedule - Urban (0000000)	226.991%

Payment Appendix
All Payer Fee Information Document YEAR 1
Fee Schedule Specifications for OTHER RADIOLOGY: as of 2/1/2025
Report Date: 12/30/2024

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Default Percent of Eligible Charges: 20.00%
Professional/Technical Modifier Pricing: Fee Source-Based
Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 = F)
Anesthesia Conversion Factor (Based on a 15 minute Anesthesia Time Unit Value): \$ 81.92
Calculation of Anesthesia Partial Units: Proration
Schedule Type: FFS

Last Routine Maintenance Update: 12-15-2024

Fixed Fees: 36415 - \$3.00 36416 - \$3.00 87804 - \$14.00

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Section 1. Definition of Terms

Unless otherwise defined in this document, capitalized terms will have the meanings ascribed to them in the Agreement.

Alternate Fee Source: The Fee Source United uses if the Primary Fee Source does not publish a Fee Basis amount. Information about United's Alternate Fee Sources can be located at www.UHCProvider.com >> Claims & Payments >> Frequently searched >> CMS 1500 Fee Sources.

AMA: American Medical Association located at: www.ama-assn.org.

Anesthesia Conversion Factor: The dollar amount that will be used in the calculation of time-based and non-time based Anesthesia Management fees in accordance with the Anesthesia Payment Policy. Unless specifically stated otherwise, the Anesthesia Conversion Factor indicated is fixed and will not change. The Anesthesia Conversion Factor is based on an anesthesia time unit value of 15 minutes. If any of United's claims systems cannot administer a 15-minute anesthesia time unit value, then the Anesthesia Conversion Factor will be calculated as follows:

$$(\text{Value of 15-minute Anesthesia Conversion Factor} / 15) * \text{anesthesia time unit value}$$

For example, an Anesthesia Conversion Factor of \$60.00 (based on a 15-minute anesthesia time unit value) would be calculated to an Anesthesia Conversion Factor of \$40.00 (based on a 10-minute anesthesia time unit value).

$$\text{Example: } (\$60.00 / 15) * 10 = \$40.00$$

Anesthesia Management: The management of anesthesia services related to medical, surgical or scopic procedures, as described in the current Anesthesia Management Codes list attached to the Anesthesia Payment Policy located at www.UHCProvider.com.

Biosimilar Product(s): A biological product that is substantially similar to a US-licensed reference biological product such that there are no clinically meaningful differences in safety, purity, and potency of the product, and only minor differences in any clinically inactive components.

Calculation of Anesthesia Partial Units:

Proration: Partial time units will be prorated and calculated to one decimal place rounded to the nearest tenth. For example, if the anesthesia time unit value is based on 15 minutes and if 17 minutes of actual time is submitted on a claim, then the 17 minutes will be divided by 15. The resulting figure of 1.1333 will be rounded to the nearest tenth and the total time units for the claim will be 1.1 time units.

If any of United's claims systems cannot administer the calculation of partial units as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the Proration method described above.

CMS: Centers for Medicare and Medicaid Services located at: www.cms.hhs.gov.

CMS Carrier Locality: CMS assigned identification number for locality specific Fee Basis Amounts.

Conversion Factor: A multiplier, expressed in dollars per relative value unit, which converts relative values into Fee Basis amounts.

Current Procedural Terminology (CPT)/Healthcare Common Procedure Coding System (HCPCS): A set of codes that describe procedures and services, including supplies and materials, performed or provided by physicians and other health care professionals. Each procedure or service is identified with a 5 digit code. The use of CPT/HCPCS simplifies the reporting of services.

CPT/HCPCS Description: The descriptor associated with each CPT/HCPCS code.

Default Percent of Eligible Charges: If a Fee Basis amount is not sourced by either a Primary Fee Source or Alternate Fee Source, such as services submitted using unlisted, unclassified or miscellaneous codes, the codes are subject to correct coding review and will be priced at the contracted percentage indicated within this document.

Expired Code: An existing CPT or HCPCS code that will be expired by the entity that published the code (for example, CMS or the AMA).

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All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Fee Amount: The contract rate for each CPT/HCPCS. The calculation of the Fee Amount is impacted by a variety of factors explained within this document including, but not limited to, Professional/Technical Modifier Pricing, Carrier Locality, CMS year, Place of Service and Pricing Level. The Fee Amount is calculated by multiplying the Fee Basis times the Pricing Level for each specific Type of Service.

Fee Basis: The amount published by the Fee Source upon which the Pricing Level will be applied to derive the Fee Amount.

Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule which supports the terms of the contractual agreement. This is the fee schedule for services performed in non-facility Places of Service.

Fee Schedule Specifications: Documentation of the underlying calculation methodology and criteria used to derive the Fee Amounts contained within the fee schedule.

Fee Source: The primary or alternate entity or publication that is supplying the Fee Basis.

Fixed Fees: Fee Amounts that are set at amounts which do not change. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Flat Rate Fee: An amount published by a Fee Source and used as a Fee Basis that is other than a RVU, such as an amount for durable medical equipment or laboratory services.

Future Payment Terms: The general description of any pricing terms which will be implemented on a scheduled future effective date.

GPCI: Geographic Practice Cost Index as adopted by CMS.

Last Routine Maintenance Update: The effective date on which this fee schedule was most recently updated. Please refer to the Routine Maintenance section of this document for more information about Routine Maintenance updates.

Linked Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule for each specific contractual agreement. This is the fee schedule for services performed in facility Places of Service.

Modifier: A Modifier provides the means to report or indicate that a service or procedure has been altered by some specific circumstance but not changed in its definition or code.

National Payment Amount: Fee Amounts calculated by United utilizing the formula adopted by CMS with no GPCIs.

Place of Service: The facility or non-facility setting in which the service is performed. This may also be referred to by CMS as Payment Type.

Pricing Level: The contracted percentage or amount that will be multiplied times the primary or alternate Fee Basis amount in order to derive the Fee Amount.

Primary Fee Source (Carrier Locality): The main Fee Source used to supply the Fee Basis amount for deriving the Fee Amount within each Type of Service category. For instance, if the Fee Amounts for a given category of codes are derived by applying a particular Pricing Level to the CMS Resource-Based Relative Value Scale (RBRVS), then CMS RBRVS is the Primary Fee Source. The Carrier Locality is designated to indicate the exact CMS geographic region upon which the Fee Amounts are based. Information about United's Primary Fee Sources can be located at www.UHCPProvider.com >> Claims & Payments >> Frequently searched >> CMS 1500 Fee Sources.

Professional/Technical Modifier Pricing: Fee Source-Based: Fee Amounts for Modifiers (for example, -TC or -26 Modifiers) are derived using the Fee Basis amount as published by the Primary Fee Source or Alternate Fee Source.

RVU: Relative Value Unit as adopted by CMS.

Replacement Code: One or more new CPT or HCPCS codes that are the exact same services or descriptions and will replace one or more Expired Codes within the same Type of Service category.

Report Date: The actual date that this document was produced.

Representative Fee Schedule Sample: A representative listing of the most commonly used CPT/HCPCS codes and fees, along with other

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

relevant pricing information, for each specific Fee Schedule ID. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Schedule Type: FFS: This is a fee-for-service fee schedule. Unless stated otherwise, the Fee Amount indicated will be used to calculate payment to you as further described within this document.

Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 =F): This fee schedule follows CMS guidelines for determining when services are priced at the facility or nonfacility fee schedule (with the exception of services performed at Ambulatory Surgery Centers, POS 24, which will be priced at the facility fee schedule). CMS guidelines can be located at: www.cms.hhs.gov.

If any of United's claims systems cannot administer the calculation of Site of Service Differential pricing as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the method described above.

Type of Service: A general categorization of related CPT/HCPCS codes. Type of Service categories are intended to closely align with the CPT groupings in the Current Procedural Terminology code book (as published by the AMA) and the HCPCS groupings (as adopted by CMS). The Office Lab Type of Service category represents those lab tests, as determined by United, in which the lab test result is necessary to make an informed treatment decision while the patient is in the office. A partial or complete crosswalk mapping of CPT/HCPCS to Type of Service categories is available to you upon request.

United: UnitedHealthcare Insurance Company or one of its affiliates which is a party to the Agreement.

Section 2. Routine Updates

Routine updates occur when United mechanically incorporates revised information created by the Fee Source, and as described below, to update the Fee Amounts calculated in accordance with this Fee Information Document. United routinely updates its fee schedule: (1) to stay current with applicable coding practices; (2) in response to price changes for immunizations and injectable medications; and (3) to remain in compliance with HIPAA requirements. United will not generally communicate routine updates of this nature.

The types of routine updates, and their respective effective dates, are described below.

a. Annual Changes to Relative Value Units, Conversion Factors, Geographical Pricing Cost Index, or Flat Rate Fees

This fee schedule follows a "stated year" construction methodology. The 2020 RVU, the 2020 Conversion Factor, the 2020 GPCI, and the 2020 Flat Rate Fee will be locked in at the end of the stated year as the basis for deriving Fee Amounts, except to add Fee Amounts for new codes. For codes that are valued using an RVU, the year of the CPT/HCPCS code initial RVU introduction along with the Conversion Factor and GPCI as stated above will be applied. If a CMS Carrier Locality is not selected the CMS national payment amount is applied. United will use reasonable commercial efforts to implement the updates in its systems on or before the later of (i) 90 days after the effective date of any modification made by the Fee Source or (ii) 90 days after the date on which the Fee Source initially places information regarding such modification in the public domain (for example, when CMS distributes program memoranda to providers). United will make the updates effective in its system on the effective date of the change by the Fee Source. However, claims already processed prior to the change being implemented by United will not be reprocessed unless otherwise required by law.

b. Quarterly Updates in Response to Changes Published by Primary Fee Source and Alternate Fee Sources

United updates its fee schedule in response to changes published by Primary Fee Sources as a result of additions, deletions, and changes to CPT codes by the AMA or HCPCS codes by CMS and any subsequent changes to CMS' annual update. United updates its fee schedules for new CPT/HCPCS codes using the applicable Conversion Factor and Pricing Level of the original construction methodology along with the then-current RVU of the published CPT/HCPCS code. The effective date of the updates described in this subsection b. will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the calendar quarter following the next calendar quarter. For example, if final publication by the Fee Source is on April 10, the fee update under this subsection b. will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee update under this subsection b. will be effective no later than October 1.

The new code pricing will be locked in at the initial pricing published by the fee source. If a CPT/HCPCS code has expired and is replaced with a Replacement Code, United will apply the Fee Amount of the Expired Code that is published by the Primary Fee Source or Alternate Fee Source to the Replacement Code.

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

c. Price Changes for Immunizations and Injectables

United routinely updates the Fee Amounts in response to price changes for immunizations and injectables published by the Fee Sources. In addition, United's Executive Drug Pricing Forum (EDPF) meets as needed to review and evaluate the drug prices that will be used in each quarterly update. The EDPF may address topics including pricing for emerging drugs, anticipated manufacturer price changes, and special circumstances (for example, H1N1 vaccine). Based on supporting information provided by the drug manufacturer or the Fee Source, United's EDPF may elect to establish a Fee Amount or override a Fee Amount, as published by the Fee Source, in favor of a Fee Amount that is more appropriate and reasonable for a particular vaccine or drug. These Fee Amount updates will be effective as described below.

For Immunizations, United applies the UHC Immunization Fee Schedule. The Centers for Disease Control and Prevention Private Sector Selling Price (CDC PSSP) is the Primary Fee Source used to obtain the Fee Basis amounts. When the Primary Fee Source does not publish a Fee Amount, United will use Alternate Fee Sources. If more than one Fee Basis amount is published by the CDC PSSP for a specific CPT/HCPCS code, an average of the published amounts will be used.

The effective date of updates under this subsection c. will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the calendar quarter following the next calendar quarter. For example, if final publication by the Fee Source is on April 10, the fee update under this subsection c. will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee update under this subsection c. will be effective no later than October 1.

d. Pricing Changes for Durable Medical Equipment (DME)

United routinely updates the Fee Amounts in response to price changes for durable medical equipment and clinical lab services published by the Fee Source.

Durable Medical Equipment (DME):

- If CMS publishes a DMEPOS floor Fee Basis amount of zero for a code and there are state based DMEPOS Fee Basis amounts for the same code, United calculates the floor based on the lowest contiguous states' non-rural fee for that code.

e. Other Updates

United reserves the right, but does not have the obligation, to perform other updates as may be necessary to (1) remain consistent with a Primary Fee Source, (2) offset any one-time or non-standard changes to a government program fee schedule, (3) implement a proportionate adjustment to reflect decreased funding in connection with a government program or (4) comply with applicable law. United will use reasonable commercial efforts to implement the updates in its systems on or before the later of (1) 90 days after the effective date of any modification made by the Fee Source or (2) 90 days after the date on which the Fee Source initially places information regarding such modification in the public domain (for example, when CMS distributes program memoranda to providers). United will make the updates effective in its system on the effective date of the change. However, claims already processed prior to the change being implemented by United will not be reprocessed unless otherwise required by law.

Section 3. Miscellaneous

Claims must be submitted using a CMS 1500, its successor form or its electronic equivalent. All claims submitted under this Payment Appendix must use CPT codes, HCPCS codes, ICD-10-CM codes or its successor and other codes in compliance with HIPAA standard data set requirements. Claims submitted without HIPAA standard data set requirements may be denied.

Fee Amounts listed in the fee schedule are all-inclusive, including without limitation any applicable taxes. Unless specifically indicated otherwise, Fee Amounts represent global fees and may be subject to reductions based on appropriate Modifier (for example, professional and technical modifiers.) As used in the previous sentence, "global fees" refers to services billed without a Modifier, for which the Fee Amount includes both the professional component and the technical component. Any co-payment, deductible or coinsurance that the customer is responsible to pay under the customer's benefit contract will be subtracted from the listed Fee Amount in determining the amount to be paid by the payer. Payer will pay claims for Covered Services according to the lesser of provider's Customary Charge or the applicable fee schedule and the actual payment amount is also subject to matters described in this agreement, such as the Payment Policies.

No payments will be made for any CMS additional compensation programs under this Payment Appendix, including without limitation value-based modifiers, incentive programs or other bonus payment programs, or payments or penalties associated with the Medicare Access and CHIP Reauthorization Act of 2015, and as amended by CMS.

Payment Appendix
All Payer Fee Information Document
Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Section 4. Services Covered or Provided by Another Program

If an applicable state, federal or other program is available to provide items or payment directly to provider for specific covered services for customers subject to this Payment Appendix that would otherwise be payable under this Payment Appendix, the applicable program will apply and not this Payment Appendix. (For example, the Vaccines For Children program currently provides vaccines free of charge, and therefore no amount will be payable under this Payment Appendix for vaccines within the Vaccines For Children program.)

For More Information United is committed to providing transparency related to our fee schedules. If you have questions about this fee schedule, please contact Network Management at the address and phone number on your contract or participation agreement. Alternatively, you may use our fee schedule look-up function on the web at: www.UHCProvider.com or contact our Voice Enabled Telephonic Self Service line at (877) 842-3210.

Payment Appendix

All Payer Fee Information Document YEAR 2

Fee Schedule Specifications for OTHER RADIOLOGY: as of 2/1/2025
Report Date: 12/30/2024

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Type Of Service Description	Primary Fee Source	Pricing Level
EVALUATION & MANAGEMENT	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
EVALUATION & MANAGEMENT - NEW PT SF MDM	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
EVALUATION & MANAGEMENT - NEW PT LOW MDM	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
EVALUATION & MANAGEMENT - NEW PT MOD MDM	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
EVALUATION & MANAGEMENT - NEW PT HIGH MDM	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
EVALUATION & MANAGEMENT - EST PT NOT REQ PHYS QHP	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
EVALUATION & MANAGEMENT - EST PT SF MDM	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
EVALUATION & MANAGEMENT - EST PT LOW MDM	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
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EVALUATION & MANAGEMENT - EST PT HIGH MDM	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
EVALUATION & MANAGEMENT - NEONATAL	2020 CMS RBRVS Carrier Locality (0000000)	149.000%
EVALUATION & MANAGEMENT - PREVENTIVE	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
EVALUATION & MANAGEMENT - NURSING FACILITY SVCS	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
E&M - EMERGENCY MEDICINE DEPARTMENT VISIT	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - INTEGUMENTARY	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - MUSCULOSKELETAL	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - RESPIRATORY	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - CARDIOVASCULAR	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - HEMIC & LYMPHATIC	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - MEDIASTINUM & DIAPHRAGM	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - DIGESTIVE	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - URINARY	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - MALE GENITAL	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - FEMALE GENITAL	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - MATERNITY & DELIVERY	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - ENDOCRINE	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - NERVOUS	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - EYE & OCULAR ADNEXA	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
SURGERY - AUDITORY	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
RADIOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
RADIOLOGY - BONE DENSITY	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
RADIOLOGY - CT	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
RADIOLOGY - MAMMOGRAPHY	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
RADIOLOGY - MRI	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
RADIOLOGY - MRA	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
RADIOLOGY - NUCLEAR MEDICINE	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
RADIOLOGY - PET SCANS	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
RADIOLOGY - RADIATION THERAPY	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
RADIOLOGY - ULTRASOUND	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
LAB - PATHOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	60.000%
OFFICE LAB	2020 CMS Clinical Lab Schedule National Limit	60.000%
CLINICAL LABORATORY	2020 CMS Clinical Lab Schedule National Limit	42.000%
MEDICINE - OPHTHALMOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	125.000%
MEDICINE - CARDIOVASCULAR	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
MEDICINE - ALLERGY & CLINICAL IMMUNOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
MEDICINE - CHIROPRACTIC MANIPULATIVE TREATMENT	2020 CMS RBRVS Carrier Locality (0000000)	90.000%
MEDICINE - PHYSICAL MED AND REHAB - MODALITIES	2020 CMS RBRVS Carrier Locality (0000000)	90.000%
MEDICINE - PHYSICAL MED AND REHAB - THERAPIES&OTHER	2020 CMS RBRVS Carrier Locality (0000000)	90.000%
MEDICINE - ENTERAL FORMULA	2020 UHC PEN NonRural Floor	40.000%
MEDICINE - OTHER	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
MEDICINE - IMMUNIZATION ADMINISTRATION	2020 CMS RBRVS Carrier Locality (0000000)	100.000%
MEDICINE - CHEMO ADMIN	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
OBSTETRICS - GLOBAL	2020 CMS RBRVS Carrier Locality (0000000)	233.003%
IMMUNIZATIONS	UHC Immunization Fee Schedule	113.000%
INJECTABLES/OTHER DRUGS	CMS Drug Pricing	100.000%
INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	CMS Drug Pricing	100.000%
INJECTABLES - IVIG	CMS Drug Pricing	100.000%
INJECTABLES-ORIGIN/BIOSIMILAR NON-ONCOLOGY DRUGS	CMS Drug Pricing	100.000%
INJECTABLES-ORIGIN/BIOSIMILAR ONCOLOGY DRUGS	CMS Drug Pricing	100.000%
INJECTABLES-SALINE & DEXTROSE SOLUTIONS	CMS Drug Pricing	100.000%
DME & SUPPLIES	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - RESPIRATORY	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - ORTHOTICS	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - PROSTHETICS	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - OSTOMY	2020 CMS DME NonRural Floor	40.000%
AMBULANCE	2020 CMS Ambulance Schedule - Urban (0000000)	233.003%

Payment Appendix
All Payer Fee Information Document YEAR 2
Fee Schedule Specifications for OTHER RADIOLOGY: as of 2/1/2025
Report Date: 12/30/2024

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Default Percent of Eligible Charges: 20.00%
Professional/Technical Modifier Pricing: Fee Source-Based
Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 = F)
Anesthesia Conversion Factor (Based on a 15 minute Anesthesia Time Unit Value): \$ 84.09
Calculation of Anesthesia Partial Units: Proration
Schedule Type: FFS

Last Routine Maintenance Update: 12-15-2024

Fixed Fees: 36415 - \$3.00 36416 - \$3.00 87804 - \$14.00

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Section 1. Definition of Terms

Unless otherwise defined in this document, capitalized terms will have the meanings ascribed to them in the Agreement.

Alternate Fee Source: The Fee Source United uses if the Primary Fee Source does not publish a Fee Basis amount. Information about United's Alternate Fee Sources can be located at www.UHCProvider.com >> Claims & Payments >> Frequently searched >> CMS 1500 Fee Sources.

AMA: American Medical Association located at: www.ama-assn.org.

Anesthesia Conversion Factor: The dollar amount that will be used in the calculation of time-based and non-time based Anesthesia Management fees in accordance with the Anesthesia Payment Policy. Unless specifically stated otherwise, the Anesthesia Conversion Factor indicated is fixed and will not change. The Anesthesia Conversion Factor is based on an anesthesia time unit value of 15 minutes. If any of United's claims systems cannot administer a 15-minute anesthesia time unit value, then the Anesthesia Conversion Factor will be calculated as follows:

$$(\text{Value of 15-minute Anesthesia Conversion Factor} / 15) * \text{anesthesia time unit value}$$

For example, an Anesthesia Conversion Factor of \$60.00 (based on a 15-minute anesthesia time unit value) would be calculated to an Anesthesia Conversion Factor of \$40.00 (based on a 10-minute anesthesia time unit value).

$$\text{Example: } (\$60.00 / 15) * 10 = \$40.00$$

Anesthesia Management: The management of anesthesia services related to medical, surgical or scopic procedures, as described in the current Anesthesia Management Codes list attached to the Anesthesia Payment Policy located at www.UHCProvider.com.

Biosimilar Product(s): A biological product that is substantially similar to a US-licensed reference biological product such that there are no clinically meaningful differences in safety, purity, and potency of the product, and only minor differences in any clinically inactive components.

Calculation of Anesthesia Partial Units:

Proration: Partial time units will be prorated and calculated to one decimal place rounded to the nearest tenth. For example, if the anesthesia time unit value is based on 15 minutes and if 17 minutes of actual time is submitted on a claim, then the 17 minutes will be divided by 15. The resulting figure of 1.1333 will be rounded to the nearest tenth and the total time units for the claim will be 1.1 time units.

If any of United's claims systems cannot administer the calculation of partial units as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the Proration method described above.

CMS: Centers for Medicare and Medicaid Services located at: www.cms.hhs.gov.

CMS Carrier Locality: CMS assigned identification number for locality specific Fee Basis Amounts.

Conversion Factor: A multiplier, expressed in dollars per relative value unit, which converts relative values into Fee Basis amounts.

Current Procedural Terminology (CPT)/Healthcare Common Procedure Coding System (HCPCS): A set of codes that describe procedures and services, including supplies and materials, performed or provided by physicians and other health care professionals. Each procedure or service is identified with a 5 digit code. The use of CPT/HCPCS simplifies the reporting of services.

CPT/HCPCS Description: The descriptor associated with each CPT/HCPCS code.

Default Percent of Eligible Charges: If a Fee Basis amount is not sourced by either a Primary Fee Source or Alternate Fee Source, such as services submitted using unlisted, unclassified or miscellaneous codes, the codes are subject to correct coding review and will be priced at the contracted percentage indicated within this document.

Expired Code: An existing CPT or HCPCS code that will be expired by the entity that published the code (for example, CMS or the AMA).

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Fee Amount: The contract rate for each CPT/HCPCS. The calculation of the Fee Amount is impacted by a variety of factors explained within this document including, but not limited to, Professional/Technical Modifier Pricing, Carrier Locality, CMS year, Place of Service and Pricing Level. The Fee Amount is calculated by multiplying the Fee Basis times the Pricing Level for each specific Type of Service.

Fee Basis: The amount published by the Fee Source upon which the Pricing Level will be applied to derive the Fee Amount.

Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule which supports the terms of the contractual agreement. This is the fee schedule for services performed in non-facility Places of Service.

Fee Schedule Specifications: Documentation of the underlying calculation methodology and criteria used to derive the Fee Amounts contained within the fee schedule.

Fee Source: The primary or alternate entity or publication that is supplying the Fee Basis.

Fixed Fees: Fee Amounts that are set at amounts which do not change. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Flat Rate Fee: An amount published by a Fee Source and used as a Fee Basis that is other than a RVU, such as an amount for durable medical equipment or laboratory services.

Future Payment Terms: The general description of any pricing terms which will be implemented on a scheduled future effective date.

GPCI: Geographic Practice Cost Index as adopted by CMS.

Last Routine Maintenance Update: The effective date on which this fee schedule was most recently updated. Please refer to the Routine Maintenance section of this document for more information about Routine Maintenance updates.

Linked Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule for each specific contractual agreement. This is the fee schedule for services performed in facility Places of Service.

Modifier: A Modifier provides the means to report or indicate that a service or procedure has been altered by some specific circumstance but not changed in its definition or code.

National Payment Amount: Fee Amounts calculated by United utilizing the formula adopted by CMS with no GPCIs.

Place of Service: The facility or non-facility setting in which the service is performed. This may also be referred to by CMS as Payment Type.

Pricing Level: The contracted percentage or amount that will be multiplied times the primary or alternate Fee Basis amount in order to derive the Fee Amount.

Primary Fee Source (Carrier Locality): The main Fee Source used to supply the Fee Basis amount for deriving the Fee Amount within each Type of Service category. For instance, if the Fee Amounts for a given category of codes are derived by applying a particular Pricing Level to the CMS Resource-Based Relative Value Scale (RBRVS), then CMS RBRVS is the Primary Fee Source. The Carrier Locality is designated to indicate the exact CMS geographic region upon which the Fee Amounts are based. Information about United's Primary Fee Sources can be located at www.UHCPProvider.com >> Claims & Payments >> Frequently searched >> CMS 1500 Fee Sources.

Professional/Technical Modifier Pricing: Fee Source-Based: Fee Amounts for Modifiers (for example, -TC or -26 Modifiers) are derived using the Fee Basis amount as published by the Primary Fee Source or Alternate Fee Source.

RVU: Relative Value Unit as adopted by CMS.

Replacement Code: One or more new CPT or HCPCS codes that are the exact same services or descriptions and will replace one or more Expired Codes within the same Type of Service category.

Report Date: The actual date that this document was produced.

Representative Fee Schedule Sample: A representative listing of the most commonly used CPT/HCPCS codes and fees, along with other

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

relevant pricing information, for each specific Fee Schedule ID. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Schedule Type: FFS: This is a fee-for-service fee schedule. Unless stated otherwise, the Fee Amount indicated will be used to calculate payment to you as further described within this document.

Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 =F): This fee schedule follows CMS guidelines for determining when services are priced at the facility or nonfacility fee schedule (with the exception of services performed at Ambulatory Surgery Centers, POS 24, which will be priced at the facility fee schedule). CMS guidelines can be located at: www.cms.hhs.gov.

If any of United's claims systems cannot administer the calculation of Site of Service Differential pricing as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the method described above.

Type of Service: A general categorization of related CPT/HCPCS codes. Type of Service categories are intended to closely align with the CPT groupings in the Current Procedural Terminology code book (as published by the AMA) and the HCPCS groupings (as adopted by CMS). The Office Lab Type of Service category represents those lab tests, as determined by United, in which the lab test result is necessary to make an informed treatment decision while the patient is in the office. A partial or complete crosswalk mapping of CPT/HCPCS to Type of Service categories is available to you upon request.

United: UnitedHealthcare Insurance Company or one of its affiliates which is a party to the Agreement.

Section 2. Routine Updates

Routine updates occur when United mechanically incorporates revised information created by the Fee Source, and as described below, to update the Fee Amounts calculated in accordance with this Fee Information Document. United routinely updates its fee schedule: (1) to stay current with applicable coding practices; (2) in response to price changes for immunizations and injectable medications; and (3) to remain in compliance with HIPAA requirements. United will not generally communicate routine updates of this nature.

The types of routine updates, and their respective effective dates, are described below.

a. Annual Changes to Relative Value Units, Conversion Factors, Geographical Pricing Cost Index, or Flat Rate Fees

This fee schedule follows a "stated year" construction methodology. The 2020 RVU, the 2020 Conversion Factor, the 2020 GPCI, and the 2020 Flat Rate Fee will be locked in at the end of the stated year as the basis for deriving Fee Amounts, except to add Fee Amounts for new codes. For codes that are valued using an RVU, the year of the CPT/HCPCS code initial RVU introduction along with the Conversion Factor and GPCI as stated above will be applied. If a CMS Carrier Locality is not selected the CMS national payment amount is applied. United will use reasonable commercial efforts to implement the updates in its systems on or before the later of (i) 90 days after the effective date of any modification made by the Fee Source or (ii) 90 days after the date on which the Fee Source initially places information regarding such modification in the public domain (for example, when CMS distributes program memoranda to providers). United will make the updates effective in its system on the effective date of the change by the Fee Source. However, claims already processed prior to the change being implemented by United will not be reprocessed unless otherwise required by law.

b. Quarterly Updates in Response to Changes Published by Primary Fee Source and Alternate Fee Sources

United updates its fee schedule in response to changes published by Primary Fee Sources as a result of additions, deletions, and changes to CPT codes by the AMA or HCPCS codes by CMS and any subsequent changes to CMS' annual update. United updates its fee schedules for new CPT/HCPCS codes using the applicable Conversion Factor and Pricing Level of the original construction methodology along with the then-current RVU of the published CPT/HCPCS code. The effective date of the updates described in this subsection b. will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the calendar quarter following the next calendar quarter. For example, if final publication by the Fee Source is on April 10, the fee update under this subsection b. will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee update under this subsection b. will be effective no later than October 1.

The new code pricing will be locked in at the initial pricing published by the fee source. If a CPT/HCPCS code has expired and is replaced with a Replacement Code, United will apply the Fee Amount of the Expired Code that is published by the Primary Fee Source or Alternate Fee Source to the Replacement Code.

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

c. Price Changes for Immunizations and Injectables

United routinely updates the Fee Amounts in response to price changes for immunizations and injectables published by the Fee Sources. In addition, United's Executive Drug Pricing Forum (EDPF) meets as needed to review and evaluate the drug prices that will be used in each quarterly update. The EDPF may address topics including pricing for emerging drugs, anticipated manufacturer price changes, and special circumstances (for example, H1N1 vaccine). Based on supporting information provided by the drug manufacturer or the Fee Source, United's EDPF may elect to establish a Fee Amount or override a Fee Amount, as published by the Fee Source, in favor of a Fee Amount that is more appropriate and reasonable for a particular vaccine or drug. These Fee Amount updates will be effective as described below.

For Immunizations, United applies the UHC Immunization Fee Schedule. The Centers for Disease Control and Prevention Private Sector Selling Price (CDC PSSP) is the Primary Fee Source used to obtain the Fee Basis amounts. When the Primary Fee Source does not publish a Fee Amount, United will use Alternate Fee Sources. If more than one Fee Basis amount is published by the CDC PSSP for a specific CPT/HCPCS code, an average of the published amounts will be used.

The effective date of updates under this subsection c. will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the calendar quarter following the next calendar quarter. For example, if final publication by the Fee Source is on April 10, the fee update under this subsection c. will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee update under this subsection c. will be effective no later than October 1.

d. Pricing Changes for Durable Medical Equipment (DME)

United routinely updates the Fee Amounts in response to price changes for durable medical equipment and clinical lab services published by the Fee Source.

Durable Medical Equipment (DME):

- If CMS publishes a DMEPOS floor Fee Basis amount of zero for a code and there are state based DMEPOS Fee Basis amounts for the same code, United calculates the floor based on the lowest contiguous states' non-rural fee for that code.

e. Other Updates

United reserves the right, but does not have the obligation, to perform other updates as may be necessary to (1) remain consistent with a Primary Fee Source, (2) offset any one-time or non-standard changes to a government program fee schedule, (3) implement a proportionate adjustment to reflect decreased funding in connection with a government program or (4) comply with applicable law. United will use reasonable commercial efforts to implement the updates in its systems on or before the later of (1) 90 days after the effective date of any modification made by the Fee Source or (2) 90 days after the date on which the Fee Source initially places information regarding such modification in the public domain (for example, when CMS distributes program memoranda to providers). United will make the updates effective in its system on the effective date of the change. However, claims already processed prior to the change being implemented by United will not be reprocessed unless otherwise required by law.

Section 3. Miscellaneous

Claims must be submitted using a CMS 1500, its successor form or its electronic equivalent. All claims submitted under this Payment Appendix must use CPT codes, HCPCS codes, ICD-10-CM codes or its successor and other codes in compliance with HIPAA standard data set requirements. Claims submitted without HIPAA standard data set requirements may be denied.

Fee Amounts listed in the fee schedule are all-inclusive, including without limitation any applicable taxes. Unless specifically indicated otherwise, Fee Amounts represent global fees and may be subject to reductions based on appropriate Modifier (for example, professional and technical modifiers.) As used in the previous sentence, "global fees" refers to services billed without a Modifier, for which the Fee Amount includes both the professional component and the technical component. Any co-payment, deductible or coinsurance that the customer is responsible to pay under the customer's benefit contract will be subtracted from the listed Fee Amount in determining the amount to be paid by the payer. Payer will pay claims for Covered Services according to the lesser of provider's Customary Charge or the applicable fee schedule and the actual payment amount is also subject to matters described in this agreement, such as the Payment Policies.

No payments will be made for any CMS additional compensation programs under this Payment Appendix, including without limitation value-based modifiers, incentive programs or other bonus payment programs, or payments or penalties associated with the Medicare Access and CHIP Reauthorization Act of 2015, and as amended by CMS.

Payment Appendix
All Payer Fee Information Document
Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Section 4. Services Covered or Provided by Another Program

If an applicable state, federal or other program is available to provide items or payment directly to provider for specific covered services for customers subject to this Payment Appendix that would otherwise be payable under this Payment Appendix, the applicable program will apply and not this Payment Appendix. (For example, the Vaccines For Children program currently provides vaccines free of charge, and therefore no amount will be payable under this Payment Appendix for vaccines within the Vaccines For Children program.)

For More Information United is committed to providing transparency related to our fee schedules. If you have questions about this fee schedule, please contact Network Management at the address and phone number on your contract or participation agreement. Alternatively, you may use our fee schedule look-up function on the web at: www.UHCProvider.com or contact our Voice Enabled Telephonic Self Service line at (877) 842-3210.

Payment Appendix

All Payer Fee Information Document YEAR 3

Fee Schedule Specifications for OTHER RADIOLOGY: as of 2/1/2025
Report Date: 12/30/2024

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Type Of Service Description	Primary Fee Source	Pricing Level
EVALUATION & MANAGEMENT	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
EVALUATION & MANAGEMENT - NEW PT SF MDM	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
EVALUATION & MANAGEMENT - NEW PT LOW MDM	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
EVALUATION & MANAGEMENT - NEW PT MOD MDM	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
EVALUATION & MANAGEMENT - NEW PT HIGH MDM	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
EVALUATION & MANAGEMENT - EST PT NOT REQ PHYS QHP	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
EVALUATION & MANAGEMENT - EST PT SF MDM	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
EVALUATION & MANAGEMENT - EST PT LOW MDM	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
EVALUATION & MANAGEMENT - EST PT MOD MDM	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
EVALUATION & MANAGEMENT - EST PT HIGH MDM	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
EVALUATION & MANAGEMENT - NEONATAL	2020 CMS RBRVS Carrier Locality (0000000)	149.000%
EVALUATION & MANAGEMENT - PREVENTIVE	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
EVALUATION & MANAGEMENT - NURSING FACILITY SVCS	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
E&M - EMERGENCY MEDICINE DEPARTMENT VISIT	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - INTEGUMENTARY	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - MUSCULOSKELETAL	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - RESPIRATORY	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - CARDIOVASCULAR	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - HEMIC & LYMPHATIC	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - MEDIASTINUM & DIAPHRAGM	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - DIGESTIVE	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - URINARY	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - MALE GENITAL	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - FEMALE GENITAL	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - MATERNITY & DELIVERY	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - ENDOCRINE	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - NERVOUS	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - EYE & OCULAR ADNEXA	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
SURGERY - AUDITORY	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
RADIOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
RADIOLOGY - BONE DENSITY	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
RADIOLOGY - CT	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
RADIOLOGY - MAMMOGRAPHY	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
RADIOLOGY - MRI	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
RADIOLOGY - MRA	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
RADIOLOGY - NUCLEAR MEDICINE	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
RADIOLOGY - PET SCANS	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
RADIOLOGY - RADIATION THERAPY	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
RADIOLOGY - ULTRASOUND	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
LAB - PATHOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	60.000%
OFFICE LAB	2020 CMS Clinical Lab Schedule National Limit	60.000%
CLINICAL LABORATORY	2020 CMS Clinical Lab Schedule National Limit	42.000%
MEDICINE - OPHTHALMOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	125.000%
MEDICINE - CARDIOVASCULAR	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
MEDICINE - ALLERGY & CLINICAL IMMUNOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
MEDICINE - CHIROPRACTIC MANIPULATIVE TREATMENT	2020 CMS RBRVS Carrier Locality (0000000)	90.000%
MEDICINE - PHYSICAL MED AND REHAB - MODALITIES	2020 CMS RBRVS Carrier Locality (0000000)	90.000%
MEDICINE - PHYSICAL MED AND REHAB - THERAPIES&OTHER	2020 CMS RBRVS Carrier Locality (0000000)	90.000%
MEDICINE - ENTERAL FORMULA	2020 UHC PEN NonRural Floor	40.000%
MEDICINE - OTHER	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
MEDICINE - IMMUNIZATION ADMINISTRATION	2020 CMS RBRVS Carrier Locality (0000000)	100.000%
MEDICINE - CHEMO ADMIN	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
OBSTETRICS - GLOBAL	2020 CMS RBRVS Carrier Locality (0000000)	239.986%
IMMUNIZATIONS	UHC Immunization Fee Schedule	113.000%
INJECTABLES/OTHER DRUGS	CMS Drug Pricing	100.000%
INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	CMS Drug Pricing	100.000%
INJECTABLES - IVIG	CMS Drug Pricing	100.000%
INJECTABLES-ORIGIN/BIOSIMILAR NON-ONCOLOGY DRUGS	CMS Drug Pricing	100.000%
INJECTABLES-ORIGIN/BIOSIMILAR ONCOLOGY DRUGS	CMS Drug Pricing	100.000%
INJECTABLES-SALINE & DEXTROSE SOLUTIONS	CMS Drug Pricing	100.000%
DME & SUPPLIES	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - RESPIRATORY	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - ORTHOTICS	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - PROSTHETICS	2020 CMS DME NonRural Floor	40.000%
DME & SUPPLIES - OSTOMY	2020 CMS DME NonRural Floor	40.000%
AMBULANCE	2020 CMS Ambulance Schedule - Urban (0000000)	239.986%

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Fee Schedule Specifications for OTHER RADIOLOGY: as of 2/1/2025
Report Date: 12/30/2024

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Default Percent of Eligible Charges: 20.00%
Professional/Technical Modifier Pricing: Fee Source-Based
Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 = F)
Anesthesia Conversion Factor (Based on a 15 minute Anesthesia Time Unit Value): \$ 86.61
Calculation of Anesthesia Partial Units: Proration
Schedule Type: FFS

Last Routine Maintenance Update: 12-15-2024

Fixed Fees: 36415 - \$3.00 36416 - \$3.00 87804 - \$14.00

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Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Section 1. Definition of Terms

Unless otherwise defined in this document, capitalized terms will have the meanings ascribed to them in the Agreement.

Alternate Fee Source: The Fee Source United uses if the Primary Fee Source does not publish a Fee Basis amount. Information about United's Alternate Fee Sources can be located at www.UHCProvider.com >> Claims & Payments >> Frequently searched >> CMS 1500 Fee Sources.

AMA: American Medical Association located at: www.ama-assn.org.

Anesthesia Conversion Factor: The dollar amount that will be used in the calculation of time-based and non-time based Anesthesia Management fees in accordance with the Anesthesia Payment Policy. Unless specifically stated otherwise, the Anesthesia Conversion Factor indicated is fixed and will not change. The Anesthesia Conversion Factor is based on an anesthesia time unit value of 15 minutes. If any of United's claims systems cannot administer a 15-minute anesthesia time unit value, then the Anesthesia Conversion Factor will be calculated as follows:

$$(\text{Value of 15-minute Anesthesia Conversion Factor} / 15) * \text{anesthesia time unit value}$$

For example, an Anesthesia Conversion Factor of \$60.00 (based on a 15-minute anesthesia time unit value) would be calculated to an Anesthesia Conversion Factor of \$40.00 (based on a 10-minute anesthesia time unit value).

$$\text{Example: } (\$60.00 / 15) * 10 = \$40.00$$

Anesthesia Management: The management of anesthesia services related to medical, surgical or scopic procedures, as described in the current Anesthesia Management Codes list attached to the Anesthesia Payment Policy located at www.UHCProvider.com.

Biosimilar Product(s): A biological product that is substantially similar to a US-licensed reference biological product such that there are no clinically meaningful differences in safety, purity, and potency of the product, and only minor differences in any clinically inactive components.

Calculation of Anesthesia Partial Units:

Proration: Partial time units will be prorated and calculated to one decimal place rounded to the nearest tenth. For example, if the anesthesia time unit value is based on 15 minutes and if 17 minutes of actual time is submitted on a claim, then the 17 minutes will be divided by 15. The resulting figure of 1.1333 will be rounded to the nearest tenth and the total time units for the claim will be 1.1 time units.

If any of United's claims systems cannot administer the calculation of partial units as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the Proration method described above.

CMS: Centers for Medicare and Medicaid Services located at: www.cms.hhs.gov.

CMS Carrier Locality: CMS assigned identification number for locality specific Fee Basis Amounts.

Conversion Factor: A multiplier, expressed in dollars per relative value unit, which converts relative values into Fee Basis amounts.

Current Procedural Terminology (CPT)/Healthcare Common Procedure Coding System (HCPCS): A set of codes that describe procedures and services, including supplies and materials, performed or provided by physicians and other health care professionals. Each procedure or service is identified with a 5 digit code. The use of CPT/HCPCS simplifies the reporting of services.

CPT/HCPCS Description: The descriptor associated with each CPT/HCPCS code.

Default Percent of Eligible Charges: If a Fee Basis amount is not sourced by either a Primary Fee Source or Alternate Fee Source, such as services submitted using unlisted, unclassified or miscellaneous codes, the codes are subject to correct coding review and will be priced at the contracted percentage indicated within this document.

Expired Code: An existing CPT or HCPCS code that will be expired by the entity that published the code (for example, CMS or the AMA).

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Additional Information About This Fee Schedule

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Fee Amount: The contract rate for each CPT/HCPCS. The calculation of the Fee Amount is impacted by a variety of factors explained within this document including, but not limited to, Professional/Technical Modifier Pricing, Carrier Locality, CMS year, Place of Service and Pricing Level. The Fee Amount is calculated by multiplying the Fee Basis times the Pricing Level for each specific Type of Service.

Fee Basis: The amount published by the Fee Source upon which the Pricing Level will be applied to derive the Fee Amount.

Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule which supports the terms of the contractual agreement. This is the fee schedule for services performed in non-facility Places of Service.

Fee Schedule Specifications: Documentation of the underlying calculation methodology and criteria used to derive the Fee Amounts contained within the fee schedule.

Fee Source: The primary or alternate entity or publication that is supplying the Fee Basis.

Fixed Fees: Fee Amounts that are set at amounts which do not change. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Flat Rate Fee: An amount published by a Fee Source and used as a Fee Basis that is other than a RVU, such as an amount for durable medical equipment or laboratory services.

Future Payment Terms: The general description of any pricing terms which will be implemented on a scheduled future effective date.

GPCI: Geographic Practice Cost Index as adopted by CMS.

Last Routine Maintenance Update: The effective date on which this fee schedule was most recently updated. Please refer to the Routine Maintenance section of this document for more information about Routine Maintenance updates.

Linked Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule for each specific contractual agreement. This is the fee schedule for services performed in facility Places of Service.

Modifier: A Modifier provides the means to report or indicate that a service or procedure has been altered by some specific circumstance but not changed in its definition or code.

National Payment Amount: Fee Amounts calculated by United utilizing the formula adopted by CMS with no GPCIs.

Place of Service: The facility or non-facility setting in which the service is performed. This may also be referred to by CMS as Payment Type.

Pricing Level: The contracted percentage or amount that will be multiplied times the primary or alternate Fee Basis amount in order to derive the Fee Amount.

Primary Fee Source (Carrier Locality): The main Fee Source used to supply the Fee Basis amount for deriving the Fee Amount within each Type of Service category. For instance, if the Fee Amounts for a given category of codes are derived by applying a particular Pricing Level to the CMS Resource-Based Relative Value Scale (RBRVS), then CMS RBRVS is the Primary Fee Source. The Carrier Locality is designated to indicate the exact CMS geographic region upon which the Fee Amounts are based. Information about United's Primary Fee Sources can be located at www.UHCPProvider.com >> Claims & Payments >> Frequently searched >> CMS 1500 Fee Sources.

Professional/Technical Modifier Pricing: Fee Source-Based: Fee Amounts for Modifiers (for example, -TC or -26 Modifiers) are derived using the Fee Basis amount as published by the Primary Fee Source or Alternate Fee Source.

RVU: Relative Value Unit as adopted by CMS.

Replacement Code: One or more new CPT or HCPCS codes that are the exact same services or descriptions and will replace one or more Expired Codes within the same Type of Service category.

Report Date: The actual date that this document was produced.

Representative Fee Schedule Sample: A representative listing of the most commonly used CPT/HCPCS codes and fees, along with other

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Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

relevant pricing information, for each specific Fee Schedule ID. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Schedule Type: FFS: This is a fee-for-service fee schedule. Unless stated otherwise, the Fee Amount indicated will be used to calculate payment to you as further described within this document.

Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 =F): This fee schedule follows CMS guidelines for determining when services are priced at the facility or nonfacility fee schedule (with the exception of services performed at Ambulatory Surgery Centers, POS 24, which will be priced at the facility fee schedule). CMS guidelines can be located at: www.cms.hhs.gov.

If any of United's claims systems cannot administer the calculation of Site of Service Differential pricing as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the method described above.

Type of Service: A general categorization of related CPT/HCPCS codes. Type of Service categories are intended to closely align with the CPT groupings in the Current Procedural Terminology code book (as published by the AMA) and the HCPCS groupings (as adopted by CMS). The Office Lab Type of Service category represents those lab tests, as determined by United, in which the lab test result is necessary to make an informed treatment decision while the patient is in the office. A partial or complete crosswalk mapping of CPT/HCPCS to Type of Service categories is available to you upon request.

United: UnitedHealthcare Insurance Company or one of its affiliates which is a party to the Agreement.

Section 2. Routine Updates

Routine updates occur when United mechanically incorporates revised information created by the Fee Source, and as described below, to update the Fee Amounts calculated in accordance with this Fee Information Document. United routinely updates its fee schedule: (1) to stay current with applicable coding practices; (2) in response to price changes for immunizations and injectable medications; and (3) to remain in compliance with HIPAA requirements. United will not generally communicate routine updates of this nature.

The types of routine updates, and their respective effective dates, are described below.

a. Annual Changes to Relative Value Units, Conversion Factors, Geographical Pricing Cost Index, or Flat Rate Fees

This fee schedule follows a "stated year" construction methodology. The 2020 RVU, the 2020 Conversion Factor, the 2020 GPCI, and the 2020 Flat Rate Fee will be locked in at the end of the stated year as the basis for deriving Fee Amounts, except to add Fee Amounts for new codes. For codes that are valued using an RVU, the year of the CPT/HCPCS code initial RVU introduction along with the Conversion Factor and GPCI as stated above will be applied. If a CMS Carrier Locality is not selected the CMS national payment amount is applied. United will use reasonable commercial efforts to implement the updates in its systems on or before the later of (i) 90 days after the effective date of any modification made by the Fee Source or (ii) 90 days after the date on which the Fee Source initially places information regarding such modification in the public domain (for example, when CMS distributes program memoranda to providers). United will make the updates effective in its system on the effective date of the change by the Fee Source. However, claims already processed prior to the change being implemented by United will not be reprocessed unless otherwise required by law.

b. Quarterly Updates in Response to Changes Published by Primary Fee Source and Alternate Fee Sources

United updates its fee schedule in response to changes published by Primary Fee Sources as a result of additions, deletions, and changes to CPT codes by the AMA or HCPCS codes by CMS and any subsequent changes to CMS' annual update. United updates its fee schedules for new CPT/HCPCS codes using the applicable Conversion Factor and Pricing Level of the original construction methodology along with the then-current RVU of the published CPT/HCPCS code. The effective date of the updates described in this subsection b. will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the calendar quarter following the next calendar quarter. For example, if final publication by the Fee Source is on April 10, the fee update under this subsection b. will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee update under this subsection b. will be effective no later than October 1.

The new code pricing will be locked in at the initial pricing published by the fee source. If a CPT/HCPCS code has expired and is replaced with a Replacement Code, United will apply the Fee Amount of the Expired Code that is published by the Primary Fee Source or Alternate Fee Source to the Replacement Code.

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c. Price Changes for Immunizations and Injectables

United routinely updates the Fee Amounts in response to price changes for immunizations and injectables published by the Fee Sources. In addition, United's Executive Drug Pricing Forum (EDPF) meets as needed to review and evaluate the drug prices that will be used in each quarterly update. The EDPF may address topics including pricing for emerging drugs, anticipated manufacturer price changes, and special circumstances (for example, H1N1 vaccine). Based on supporting information provided by the drug manufacturer or the Fee Source, United's EDPF may elect to establish a Fee Amount or override a Fee Amount, as published by the Fee Source, in favor of a Fee Amount that is more appropriate and reasonable for a particular vaccine or drug. These Fee Amount updates will be effective as described below.

For Immunizations, United applies the UHC Immunization Fee Schedule. The Centers for Disease Control and Prevention Private Sector Selling Price (CDC PSSP) is the Primary Fee Source used to obtain the Fee Basis amounts. When the Primary Fee Source does not publish a Fee Amount, United will use Alternate Fee Sources. If more than one Fee Basis amount is published by the CDC PSSP for a specific CPT/HCPCS code, an average of the published amounts will be used.

The effective date of updates under this subsection c. will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the calendar quarter following the next calendar quarter. For example, if final publication by the Fee Source is on April 10, the fee update under this subsection c. will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee update under this subsection c. will be effective no later than October 1.

d. Pricing Changes for Durable Medical Equipment (DME)

United routinely updates the Fee Amounts in response to price changes for durable medical equipment and clinical lab services published by the Fee Source.

Durable Medical Equipment (DME):

- If CMS publishes a DMEPOS floor Fee Basis amount of zero for a code and there are state based DMEPOS Fee Basis amounts for the same code, United calculates the floor based on the lowest contiguous states' non-rural fee for that code.

e. Other Updates

United reserves the right, but does not have the obligation, to perform other updates as may be necessary to (1) remain consistent with a Primary Fee Source, (2) offset any one-time or non-standard changes to a government program fee schedule, (3) implement a proportionate adjustment to reflect decreased funding in connection with a government program or (4) comply with applicable law. United will use reasonable commercial efforts to implement the updates in its systems on or before the later of (1) 90 days after the effective date of any modification made by the Fee Source or (2) 90 days after the date on which the Fee Source initially places information regarding such modification in the public domain (for example, when CMS distributes program memoranda to providers). United will make the updates effective in its system on the effective date of the change. However, claims already processed prior to the change being implemented by United will not be reprocessed unless otherwise required by law.

Section 3. Miscellaneous

Claims must be submitted using a CMS 1500, its successor form or its electronic equivalent. All claims submitted under this Payment Appendix must use CPT codes, HCPCS codes, ICD-10-CM codes or its successor and other codes in compliance with HIPAA standard data set requirements. Claims submitted without HIPAA standard data set requirements may be denied.

Fee Amounts listed in the fee schedule are all-inclusive, including without limitation any applicable taxes. Unless specifically indicated otherwise, Fee Amounts represent global fees and may be subject to reductions based on appropriate Modifier (for example, professional and technical modifiers.) As used in the previous sentence, "global fees" refers to services billed without a Modifier, for which the Fee Amount includes both the professional component and the technical component. Any co-payment, deductible or coinsurance that the customer is responsible to pay under the customer's benefit contract will be subtracted from the listed Fee Amount in determining the amount to be paid by the payer. Payer will pay claims for Covered Services according to the lesser of provider's Customary Charge or the applicable fee schedule and the actual payment amount is also subject to matters described in this agreement, such as the Payment Policies.

No payments will be made for any CMS additional compensation programs under this Payment Appendix, including without limitation value-based modifiers, incentive programs or other bonus payment programs, or payments or penalties associated with the Medicare Access and CHIP Reauthorization Act of 2015, and as amended by CMS.

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Section 4. Services Covered or Provided by Another Program

If an applicable state, federal or other program is available to provide items or payment directly to provider for specific covered services for customers subject to this Payment Appendix that would otherwise be payable under this Payment Appendix, the applicable program will apply and not this Payment Appendix. (For example, the Vaccines For Children program currently provides vaccines free of charge, and therefore no amount will be payable under this Payment Appendix for vaccines within the Vaccines For Children program.)

For More Information United is committed to providing transparency related to our fee schedules. If you have questions about this fee schedule, please contact Network Management at the address and phone number on your contract or participation agreement. Alternatively, you may use our fee schedule look-up function on the web at: www.UHCProvider.com or contact our Voice Enabled Telephonic Self Service line at (877) 842-3210.